Presidency University, Kolkata

An Evaluation of the Swasthya Sathi Scheme in West Bengal

by
Ritobhash Daw
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Introduction

Swasthya Sathi is a health insurance scheme introduced by the Government of West Bengal in 2016 to promote affordable healthcare access, initially targeting economically vulnerable groups and later expanded in 2020 to include all residents of West Bengal. The scheme provides cashless, paperless healthcare services through *Swasthya Sathi Smart Cards*, covering up to ₹5 lakh per family annually for treatments including high-cost procedures, pre-existing conditions, and maternity care. Beneficiaries must be permanent residents of West Bengal, and the family unit covered under the scheme is defined to include the beneficiary, spouse, parents of both spouses, sons up to 18 years of age, and unmarried daughters up to 21 years of age. With the scheme covering over 1,800 medical procedures including critical illnesses, surgeries, and diagnostic services, *Swasthya Sathi* currently operates through a network of 2688 public and private hospitals across the state.

Research Aim

The aim of this study is to evaluate the effectiveness of the *Swasthya Sathi* scheme in improving healthcare accessibility and reducing the financial burden of hospitalisation among residents of West Bengal by assessing its implementation, awareness, accessibility, and satisfaction among users.

Research Objective

The objectives of this study are:

1. Assess awareness levels about the *Swasthya Sathi* Scheme's benefits among enrolled families.
2. Examine enrolment rates and the demographic factors influencing scheme participation.
3. Analyse users' experiences with the hospitalisation process under the scheme, including satisfaction with covered services and perceived accessibility improvements.
4. Identify key barriers to enrolment and usage, such as lack of awareness or perceived quality of care.

Method

The present study employed a survey-based approach to assess the effectiveness of the *Swasthya Sathi* scheme in West Bengal.

Participants

A total of 129 participants were interviewed from Kolkata, Howrah, and nearby districts in West Bengal. Eligible participants were required to have at least one family member hospitalised within the past two years. While the study primarily targeted female cardholders of the *Swasthya Sathi* scheme as respondents, male family members were permitted to participate in cases where the female cardholder was unavailable. This alternative was allowed only when the male respondent was present during the scheme's enrolment process and possessed comprehensive knowledge about it.

Materials

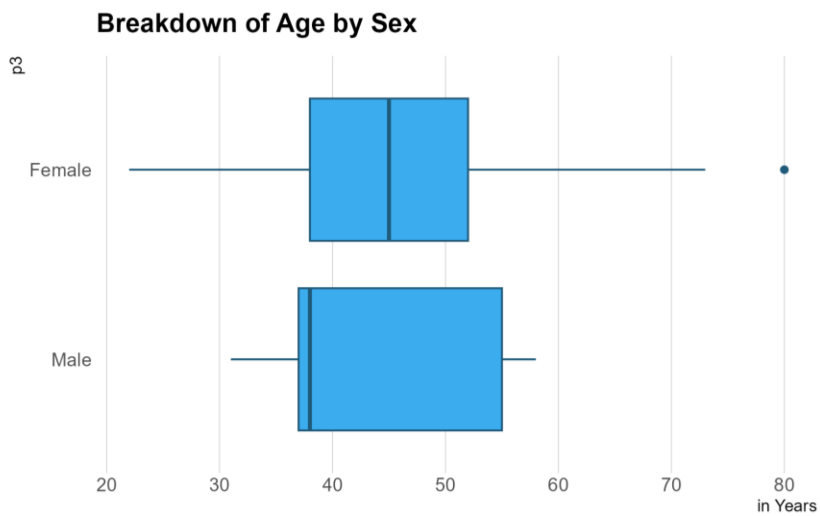
Data were collected using a structured questionnaire specifically designed for this study. The questionnaire investigated multiple dimensions of the *Swasthya Sathi* scheme, including the enrolment process, details of hospitalisation episodes, and the practical utility of the *Swasthya Sathi* card. Additionally, it assessed public awareness of the scheme and examined how the financial assistance provided through the scheme influenced participants' medical decision-making capabilities. It was pre-tested to ensure clarity and relevance.

Procedure

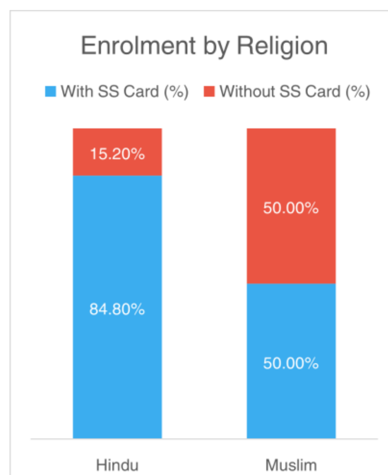
Face-to-face interviews were conducted between August and September 2024. Participants were individually approached, and responses were recorded using a paper-and-pen format. Ethical considerations were upheld throughout, including informed consent and confidentiality.

Profile of the Respondents

Of the total 129 respondents, 28 were residents of Kolkata, 88 were from Howrah and suburban areas, and 13 were from other districts in West Bengal. A total of 124 were women while the 5 were men. In terms of *Swasthya Sathi* enrolment rates, 84% of the women were card-holders, while 80% of men replied to being a beneficiary of the same. Analysis of enrolment by sex is less relevant, as male family members cannot independently register. When it comes to age, the median for female respondents was 45, while it was 38 for male respondents. We find that there is no statistically significant difference in the average age between those who enrolled and those who did not.[1]



A breakdown by religious affiliation shows that 125, or 97% of the sample were Hindu, while only 4, or 3%, were Muslim. The enrolment rates among Hindu respondents were relatively higher at 85%, while exactly half of the Muslim respondents were enrolled. We find a statistically significant difference in enrolment rates between two groups.[2] Further, we also find that religion has a significant bearing on whether one enrols into the scheme, with Hindu respondents having higher enrolment probabilities than their Muslim counterparts.[3]

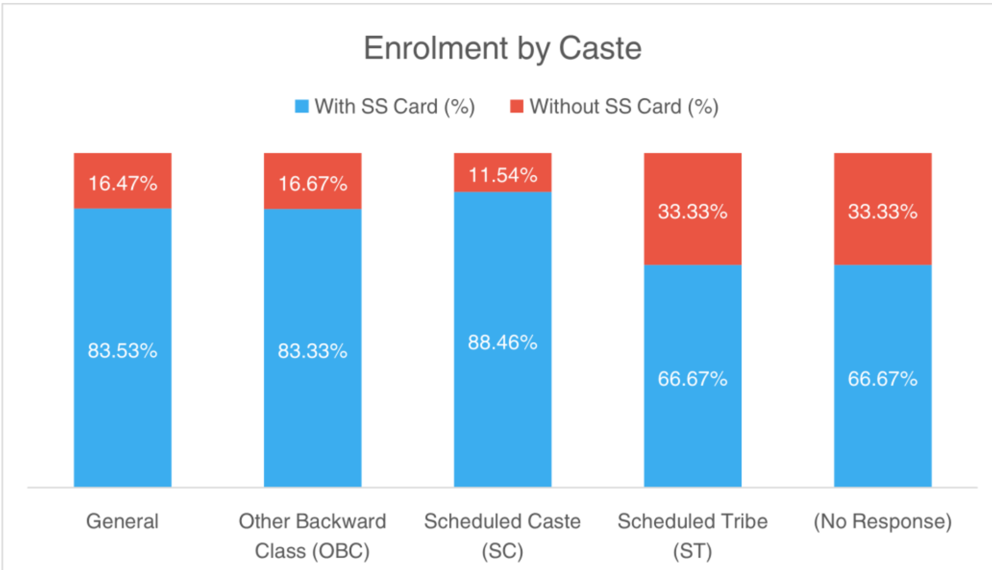


[1] Welch Two Sample t-test is insignificant at 10% level.

[2] Kruskal-Wallis rank sum test is significant at 10% level. Results have been summarised in Table B1.

[3] Logistic regression is significant at 10% level. Results have been summarised in Table B2.

In terms of caste, 66% belonged to the General category, while 9%, 20% and 2% belonged to Other Backward Classes (OBC), Scheduled Caste (SC) and Scheduled Tribe (ST). With the exception of Scheduled Tribe (STs), enrolment rates by caste were more-or-less similar at 86%. Though we find a statistically insignificant difference in enrolment rates among the caste categories[4], it should be noted that the number of non-General-and-OBC respondents are not sufficiently representative.

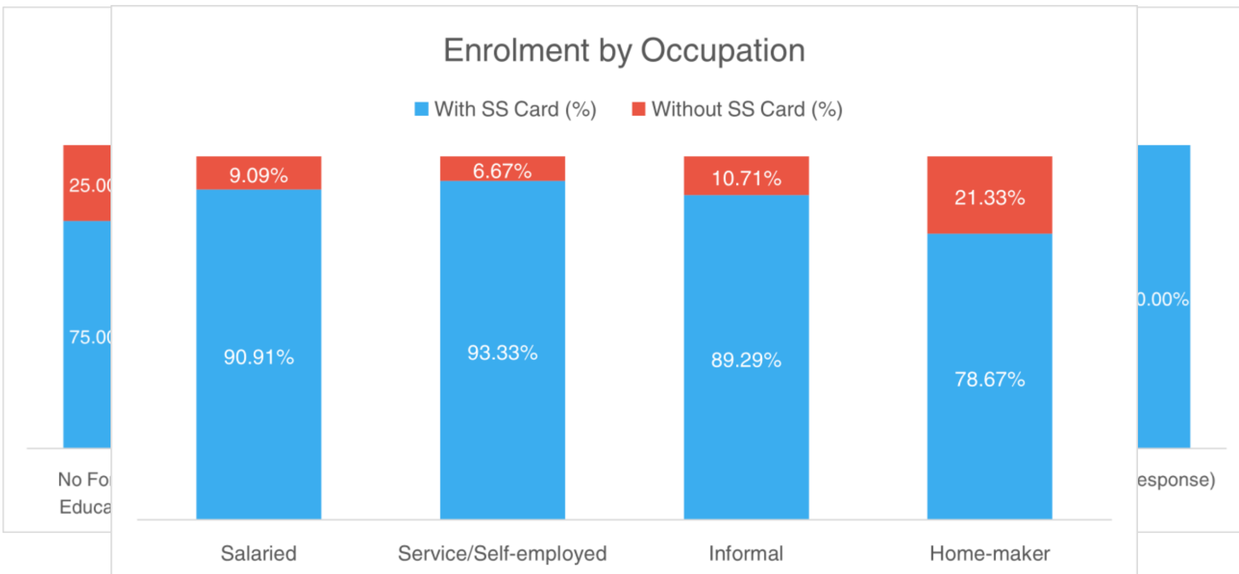


As for the educational qualifications of the interviewees, 6% reported having no formal education, while 9% of the sample reported not having completed their primary education. 21%, 31% and 15% had completed primary, secondary and higher-secondary education respectively, while graduates and postgraduates were 15% and 2% of the distribution. Enrolment rates were at its highest amongst those who have completed at least some level of schooling, at around 90%. Graduates and

[4] Kruskal-Wallis rank sum test is insignificant at 10% level.

postgraduates had much lower enrolment rates at 64% and 50% respectively, while respondents with little-to-no schooling enrolled at 75%.

Occupational distribution-wise, a majority of respondents were either unemployed or home-makers (58%). 22% of the respondents were engaged in the informal sector while only 12% were working in the service sector or are self-employed. Of the 11 individuals (8% of the sample) who belonged to the salaried class, 9 of them were women. Barring home-makers and those who are unemployed—79% of whom are beneficiaries—enrolment rates were very high at about 90%.



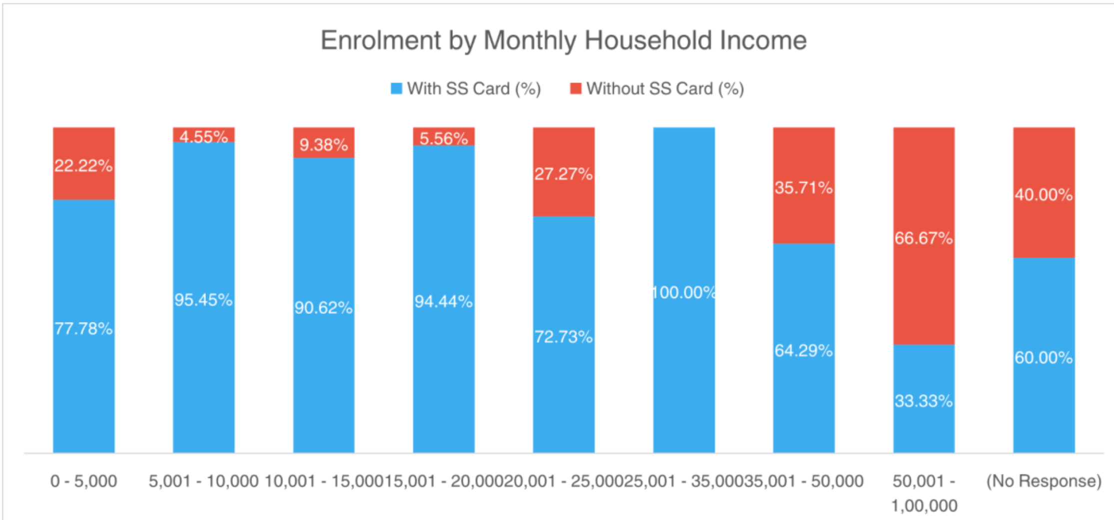
Analysis of the distribution of household income tells us that enrolment rates drop starkly once households cross the Rs. 35,000 threshold. We also find an overall statistically significant difference in enrolment rates between income groups.[5] Further, enrolment rates also varied between Below Poverty Line (BPL) card-holders and non-holders. 95% of BPL card-holders, who were 31% of our sample, were enrolled in the scheme, while 77% of non-holders, who were 65% of our sample, were enrolled in the scheme. We not only find that the difference is statistically significant[6], but also that

[5] Kruskal-Wallis rank sum test is significant at 1% level. Results have been summarised in Table B4.

[6] Kruskal-Wallis rank sum test is significant at 5% level. Results have been summarised in Table B4.

BPL card ownership significantly affects enrolment rates, with card-holders much more likely to be enrolled in the scheme.[7]

Family size among respondents were fairly representative: 39% of the respondents had less than four members, 33% had exactly four members while 28% had more than four members. Consistent with

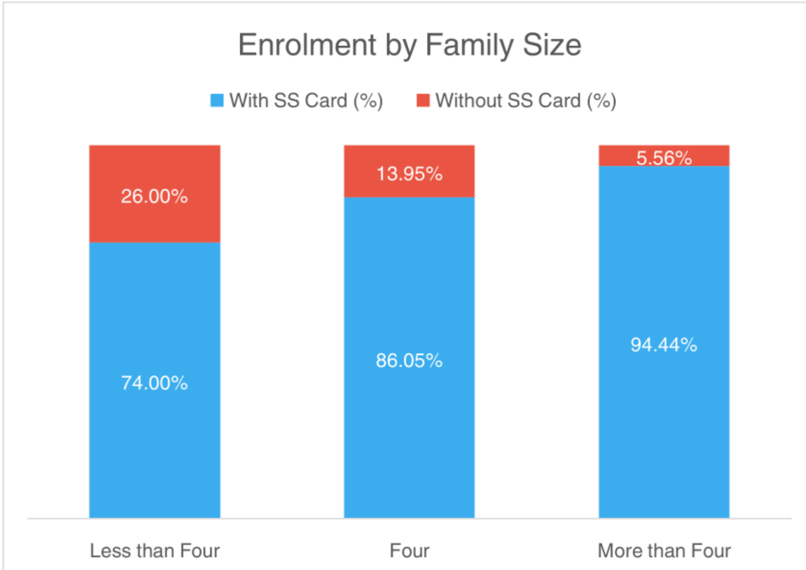


our intuition, interviewees with larger families enrolled at much higher rates. As we find, families with more than four family members have a significantly higher likelihood of enrolling, while those with exactly four members are just as likely to enrol as their less-than-four counterparts.[8] As for the number of aged members in their families, 59% reported having none; 35% had a single aged member, while 6% had two or more. Interestingly, this number does not have a bearing on whether an individual (or their family) chooses to enrol or not.[9]

[7] Logistic regression shows that BPL card-holders have a significant association with enrolment outcome at 5% level, while non-holders are significant at 1% level. Results have been summarised in Table B5.

[8] Logistic regression is significant for families with more than four members and insignificant for those with exactly four members at 5% level. Results have been summarised in Table B6.

[9] Logistic regression is insignificant for individuals with both one or two-or-more members at 10% level.



Along the same lines, enrolment rates also vary significantly based on the number of earning members in the family: those with at most one, who were 57% of our sample, enrolled at a 75% rate, while those with two or more, who were the remaining 43%, enrolled at a 95% rate - a statistically significant difference.[10]

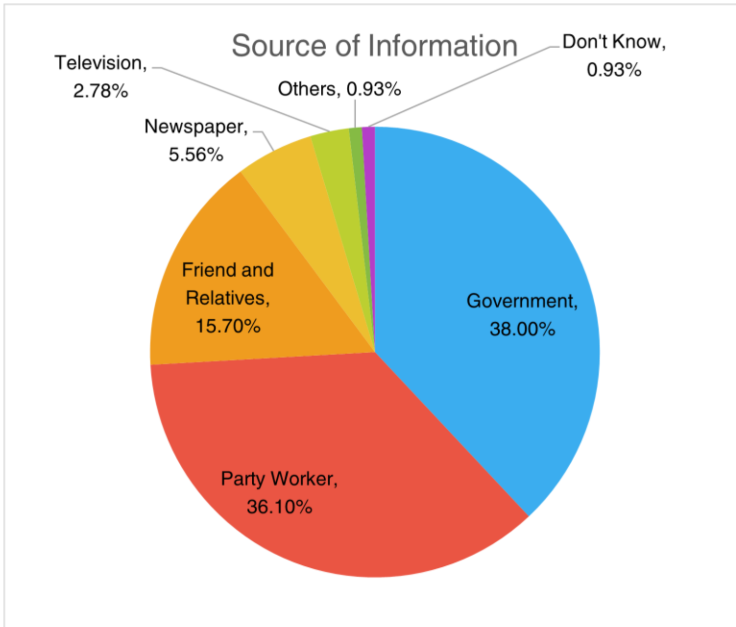
Enrolment & Awareness

Process of Enrolment

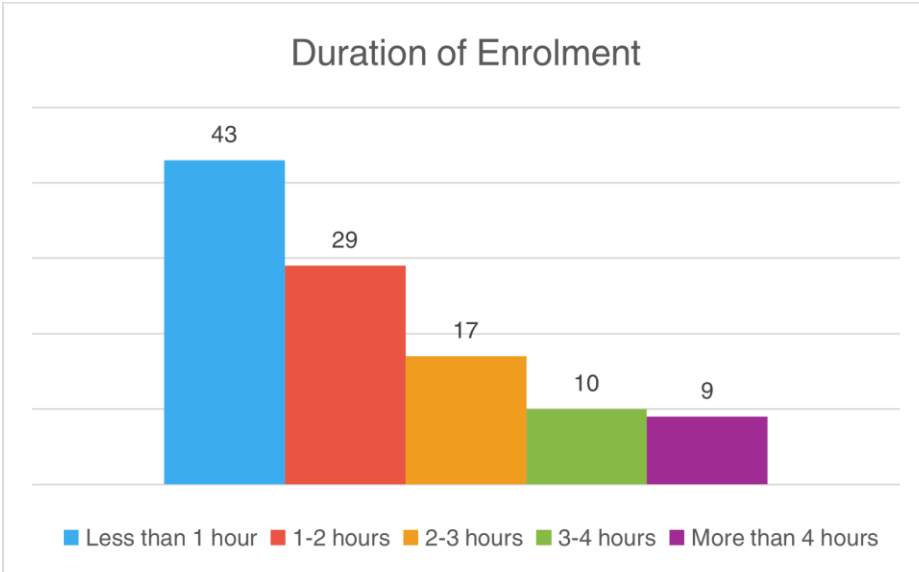
Respondents recounted Government Campaigns (38%) and Party Workers (36.1%) as their primary sources of information on the scheme, followed by Friends, Family & Relatives (15.7%), Newspaper (5.6%), Television (2.8%) as well as Other sources (1.9%). A significant portion of enrollees enrolled themselves through *Duare Sarkar* (97%), while the rest opted for Online or Mobile App registration.

[10] Kruskal-Wallis rank sum test is significant at 1% level. Results have been summarised in Table B7.

Direct efforts of the government in spreading awareness through campaigns and party workers, as well as in accessibility through outreach programs have thus proven substantial.
About 2 in 7 reported requiring no assistance during the enrolment process, while Party Workers (39%), followed by Government Officers (22%) assisted when required. Family photos and biometric scans were taken in 89% and 94% of the cases respectively, with over 90% of the respondents reporting no significant issues with either.



As for duration, respondents replied having taken on average two hours to enrol, with around 40% completing the process in less than an hour, but a bit over 8% taking more than four hours. It took on average 3 months for beneficiaries to receive their cards. Alarmingly, it took a non-negligible fraction (5%) upwards of two years to receive the same. Questions on the efficiency of the bureaucratic process remain ever-pertinent.



Family attitude turned out to be a significant motivator behind enrolment, with 44% of the spouses of respondents being actively interested in availing the scheme. 38%, however, remained indifferent.

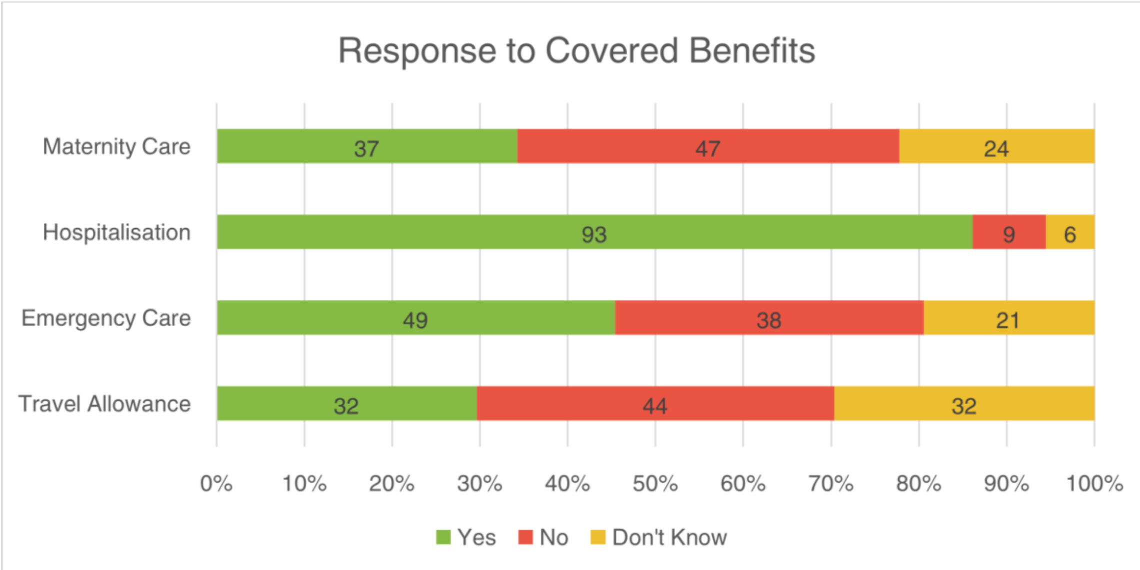
Awareness about the Scheme

To measure the respondents' awareness of the scheme was assessed through seven factual and non-factual statements:

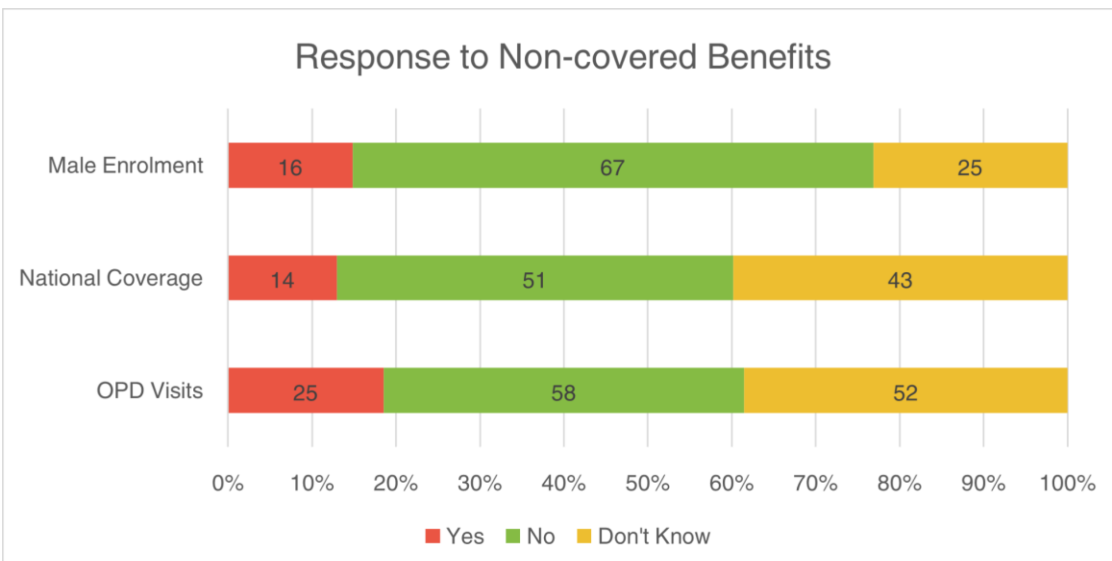
1. Eligibility for a travel allowance of Rs 200 per hospital visit
2. Coverage of emergency care costs
3. Coverage of hospitalisation costs
4. Coverage of maternity and newborn care costs
5. Coverage of Outpatient Department (OPD) visit costs
6. Usability of the Swasthya Sathi Card outside West Bengal
7. Eligibility for a husband, son, or father to register under the Swasthya Sathi scheme

Each statement required a 'Yes,' 'No,' or 'Don't know' answer.

Statements 1 to 4 are factually correct, and hence a 'Yes' response not only reflects their awareness of the true provisions of the scheme but also their confidence in confirming the same. Barring statements 1 and 4, a plurality of respondents demonstrated accurate awareness. Awareness about provisions for travel allowances was found to be the least, with only 2 in 7 respondents answering the statement correctly.



Statements 5 to 7 are factually incorrect, and a 'No' response indicates awareness of the scheme's actual provisions and confidence in refuting inaccuracies. This time, without exceptions, a plurality of respondents demonstrated accurate awareness.



We notice a worrying trend where a sizable fraction could not confidently confirm or deny most statements. Statements 1, 4, and 6 especially exemplify this trend, with respondents answering 'Don't Know' in 30%, 44%, and 40% of the cases respectively.

Reasons for Non-enrolment[11]

Of the 21 interviewees who opted not to enrol, 53% cited "Concerns about the quality of care" as the primary reason for non-enrolment. This was followed by 47% citing non-requirement of Government health insurance, while another 47% were dissatisfied towards the coverage of empanelled hospitals and physicians. These findings suggest scepticism about the state's capacity to provide quality healthcare, prompting many to opt in favour of private insurance. About 29% of the group found the enrolment process to be difficult, suggesting a need for a revaluation and simplification of the enrolment process. Finally, 29% were already registered under some other government health scheme, disqualifying them from availing the scheme altogether. At least one of the interviewees were covered under the Central Government Health Scheme (CGHS).

Experience

Hospitalisation Episode[12]

Among the respondents, 54% reported utilising their *Swasthya Sathi* cards during their most recent hospitalisation episode. A priori, we hypothesise that individuals who used their cards will have shorter time gaps between the onset of symptoms and diagnosis, as well as between diagnosis and hospitalisation, compared to those who did not. We, however, find no such evidence to support this: the time gap between the two groups in both of these cases shows no statistically significant difference. On a similar vein, we also find no evidence to support the claim that those with the card may be tended to earlier than those without: the waiting time for admission between the two groups was again not significantly dissimilar. Finally, the difference in delay due to lack of beds between the two groups was also found to be statistically insignificant.[13] [14]

To evaluate the respondents' experiences with hospitalisation, they were asked to rate their satisfaction on a three-point Likert scale ('Good,' 'Okay,' 'Bad') regarding the following aspects:

1. Admission process
2. Treatment process
3. Care taken by doctors

[11] A detailed chart of all reasons is presented in Figure C5.

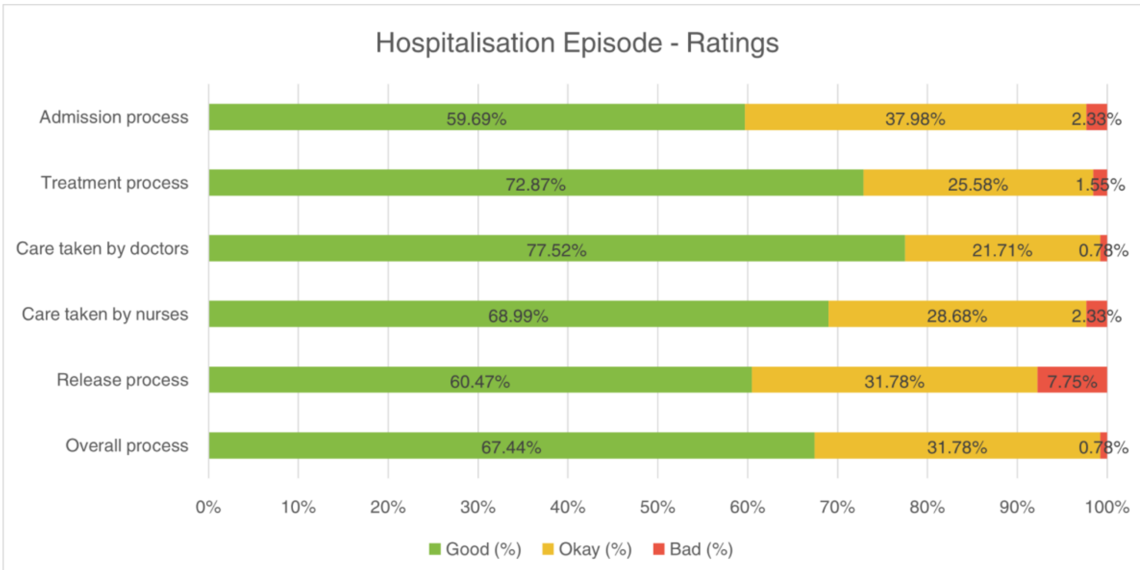
[12] Additional data on hospitalisation episodes is provided in Table C1.

[13] Shapiro-Wilk tests typically showed non-normality of the variables at 10% level, ruling out t-tests.

[14] Kruskal-Wallis rank sum test is insignificant for differences in time gap between symptoms and diagnosis, diagnosis and hospitalisation and a delay due to lack of beds between the two groups at 10% level.

4. Care taken by nurses
5. Release process
6. Overall process

The findings indicate that respondents were satisfied with the logistics of their episode. Moreover, we also fail to find any statistically significant difference in ratings between the two aforementioned groups.[15]



Experience with Swasthya Sathi

Among those who used their *Swasthya Sathi* cards during the most hospitalisation episode, a little over 90% recounted having a positive experience with it. Most respondents (80%) felt hospitals provided clear and sufficient admission information, though 23% found the documentation requirements excessive. Worryingly, we find that around 30% of *Swasthya Sathi* cardholders were improperly asked to deposit a fee, despite no legal requirement for such charges under the scheme. Some respondents, however, noted they were not particularly concerned, as the scheme has already covered the substantially higher treatment costs.

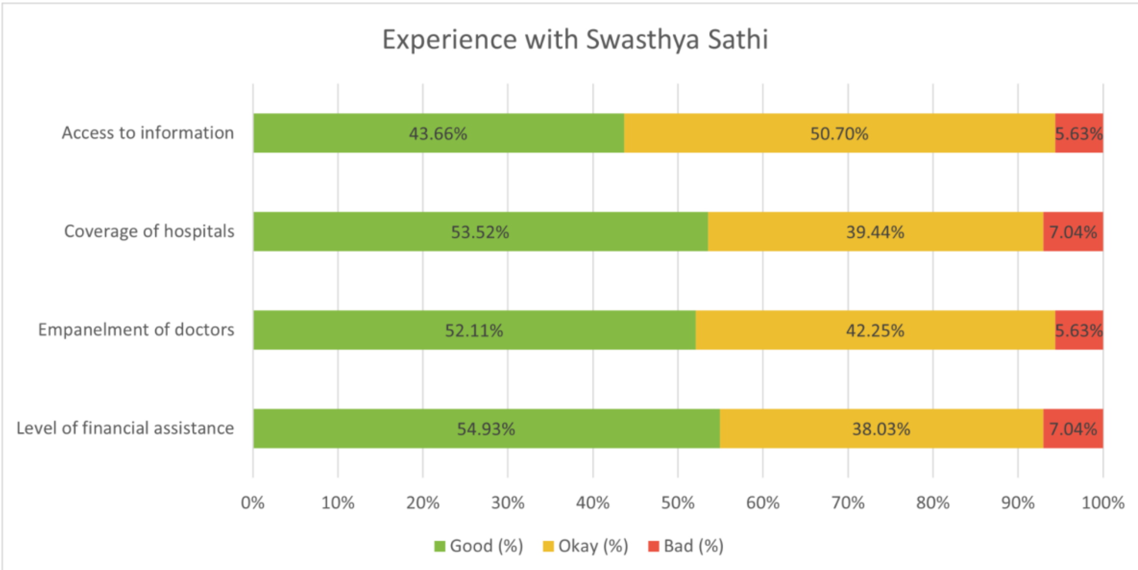
The group of respondents who utilised their cards were moreover asked to rate their level of satisfaction on a three-point Likert scale regarding the following aspects of the scheme:

1. Access to information
2. Coverage of hospitals

[15] Kruskal-Wallis rank sum test is insignificant at 10% level.

3. Empanelment of doctors
4. Level of financial assistance

We find that respondents overall were overwhelmingly satisfied (comprising 'Good' and 'Okay' ratings) with the quality of service provided by the scheme. However, the consistent 5-7% dissatisfaction rate among card-users remains a cause for concern.



Reasons for not using the Swasthya Sathi card[16]

Card-holders, who opted not to utilise their benefits, popularly cited the non-empanelment of their chosen hospitals or physicians as their reason at 70%. Given its recurrence, the government must immediately focus on improving its coverage of facilities and medical staff. Following, in 43% of the cases, respondents picked not being aware of the benefits of the scheme as well as simply not remembering about the card itself as their reasons. To avoid situations like this, the government must issue more frequent PSAs (Public Service Announcements), as well as actively engage with the patients (or those on behalf of them) and assist them more readily at hospitals. Finally, 16% responded having utilised the health insurance they already possessed.

[16] A detailed chart of all reasons is presented in Figure C6.

Status of Respondent

Finally, interviewees who enrolled into the scheme were asked to rate their likelihood of discussing health-related issues following their enrolment. Among those who used the card in their most recent hospitalisation, 63% selected 'Not at all likely' to 'Somewhat likely': there has been little-to-no change in the frequency of discussion on this topic. On the other hand, interestingly, among those who did not use the card, 42% selected 'Very likely' and 'Almost always': there has been an increase in discussions on the topic. Between these two groups, we find a statistically significant difference in their ratings.[17] Further, 9% of those who used their cards and 19% those who did not were uncertain about the change in frequency of discussion. We discern that a moral hazard situation may be at play here, with card-users appearing to have a lower concern for their and their family's health than their non-card-user counterparts.

Respondents were finally asked whether they felt they had better access to health care facilities after their enrolment into the scheme. Among card-users, 59% responded positively, while 26% were uncertain. On the other hand, among non-card-users, only 29% responded positively. 32% felt indifferent while almost 40% were uncertain. We, however, fail to find a statistically significant difference between the groups. [18]

Our analysis therefore indicates a significant correlation between the use of the scheme and an improvement in the respondents' healthcare accessibility. Conversely, non-users displayed limited understanding of the scheme and its provisions and benefits.

Conclusion

The findings of this study provide a comprehensive evaluation of the *Swasthya Sathi* scheme's impact on healthcare accessibility and financial relief in West Bengal. Overall, the scheme demonstrates significant potential to improve healthcare outcomes, though certain structural and operational limitations persist.

Enrollment rates were high among vulnerable groups, such as Below Poverty Line (BPL) families and larger households, indicating the scheme's relative success in targeting economically weaker populations. However, disparities in enrollment were observed across religious and educational lines,

[17] Kruskal-Wallis rank sum test is significant at 1% level. Results have been summarised in Table B8.

[18] Kruskal-Wallis rank sum test is insignificant at 10% level.

with lower participation among Muslim respondents and highly educated individuals. This suggests a need for more inclusive outreach and trust-building efforts, particularly in underrepresented communities.

Awareness of the scheme's provisions varied widely. While a majority of respondents understood core benefits such as hospitalisation and maternity care coverage, knowledge of specific provisions, like travel allowances and card usability outside West Bengal, was notably lacking. As such, clearer communication strategies and consistent public engagement are an imperative.

User experiences were largely positive among those who utilised the scheme, with high satisfaction reported for both the financial assistance provided and the quality of hospital services. Nevertheless, issues such as improper demands for additional fees and limited empanelment of preferred hospitals emerged as recurring challenges. These, along with concerns about the quality of care, were primary reasons cited by non-enrollees and non-users, revealing areas where the scheme's implementation could be improved.

Interestingly, while the scheme appeared to improve perceived access to healthcare among card-users, non-users remained sceptical or unaware of its benefits. The discrepancy highlights the critical role of sustained government efforts to not only expand the scheme's reach but also ensure its accessibility translates into real, tangible improvements in healthcare outcomes.

In conclusion, while the *Swasthya Sathi* Scheme has brought West Bengal closer to universal healthcare, its success remains uneven. Greater focus on addressing gaps in awareness, improving delivery of service, and targeted efforts to include disadvantaged groups in its implementation will be crucial for the scheme to achieve its full potential.

Appendix A. *Demographic Breakdown of the Respondents*

Characteristics	With SS Card (%)	Without SS Card (%)	Total (%)
Sex			
Male	80.00%	20.00%	5 (3.88%)
Female	83.87%	16.13%	124 (96.12%)
Religion			
Hindu	84.80%	15.20%	125 (96.90%)
Muslim	50.00%	50.00%	4 (3.10%)
Caste			
General	83.53%	16.47%	85 (65.89%)
Other Backward Class (OBC)	83.33%	16.67%	26 (20.16%)
Scheduled Caste (SC)	88.46%	11.54%	13 (10.08%)
Scheduled Tribe (ST)	66.67%	33.33%	12 (9.30%)
(No Response)	100.00%	0.00%	3 (2.33%)
Marital Status			
Married	83.48%	16.52%	115 (89.15%)

Characteristics	With SS Card (%)	Without SS Card (%)	Total (%)
Widowed	90.00%	10.00%	10 (7.75%)
Unmarried	66.67%	33.33%	3 (2.33%)
Separated	100.00%	0.00%	1 (0.78%)

Educational Qualification			
No Formal Education	75.00%	25.00%	8 (6.20%)
Below Primary	75.00%	25.00%	12 (9.30%)
Completed Primary	92.59%	7.41%	27 (20.93%)
Secondary	90.00%	10.00%	40 (31.01%)
Higher Secondary	89.47%	10.53%	19 (14.73%)
Graduate	63.16%	36.84%	19 (14.73%)
Postgraduate	50.00%	50.00%	2 (1.55%)
(No Response)	100.00%	0.00%	2 (1.55%)
Family Size			
Less than Four	74.00%	26.00%	50 (38.76%)
Four	86.05%	13.95%	43 (33.33%)

More than Four	94.44%	5.56%	36 (27.91%)
Aged Members			
None	84.21%	15.79%	76 (58.91%)
One	84.44%	15.56%	45 (34.88%)
Two or more	75.00%	25.00%	8 (6.20%)
Occupation			
Salaried	90.91%	9.09%	11 (8.53%)
Service/Self-employed	93.33%	6.67%	15 (11.63%)
Informal	89.29%	10.71%	28 (21.71%)
Home-maker	78.67%	21.33%	75 (58.14%)
Household Income			
₹0 - ₹5,000	77.78%	22.22%	9 (6.98%)
₹5,001 - ₹10,000	95.45%	4.55%	22 (17.05%)
₹10,001 - ₹15,000	90.62%	9.38%	32 (24.81%)
₹15,001 - ₹20,000	94.44%	5.56%	18 (13.95%)
₹20,001 - ₹25,000	72.73%	27.27%	11 (8.53%)
₹25,001 - ₹35,000	100.00%	0.00%	12 (9.30%)

₹35,001 - ₹50,000	64.29%	35.71%	14 (10.85%)
₹50,001 - ₹1,00,000	33.33%	66.67%	6 (4.65%)
(No Response)	60.00%	40.00%	5 (3.88%)
Earning Members			
At most one	75.34%	24.66%	73 (56.59%)
Two or more	94.64%	5.36%	56 (43.41%)
BPL Cardholders			
Yes	95.00%	5.00%	40 (31.01%)
No	77.38%	22.62%	84 (65.12%)
(No Response)	100.00%	0.00%	5 (3.88%)

Appendix B. Statistical Tests & Analysis

Note: Coefficients in Logistic Regression summaries report the log-odds; the corresponding standard errors have been presented in brackets.

Table B1

Differences in Enrolment Rates by Religion: Kruskal-Wallis Test Results

Enrolled	Obs	Rank Sum
No	21	1452.00
Yes	108	6933.00

χ^2 (with ties) $= 3.417$, $df = 1$, $p = 0.0645^*$

Analysis reveals significant differences in enrolment rates between Hindu and Muslim respondents.

Table B2

Likelihood of Enrolment by Religious Affiliation: Logistic Regression Results

	Dependent variable:
	Enrolment
Religion	
Muslim	-1.719^*
	(1.031)
Constant	1.719^{***}
	(0.249)
Observations	129
Log Likelihood	-56.043
Akaike Inf. Crit.	116.086
Note:	*p<0.1; **p<0.05; ***p<0.01

A Muslim individual shows significantly lower odds of enrolment ($\beta = -1.719$, $p < 0.1$) compared to their Hindu counterparts, with baseline enrolment probability being positive ($\beta = 1.719$, $p < 0.1$).

Table B3

Differences in Enrolment Rates by Household Income: Kruskal-Wallis Test Results

Enrolled	Obs	Rank Sum
No	21	1804.00
Yes	108	6581.00

χ^2 (with ties) $= 8.054$, $df = 1$, $p = 0.0045^{***}$

Analysis reveals significant differences in enrolment rates across income groups ($p < 0.01$).

Table B4

Differences in Enrolment Rates by BPL Status: Kruskal-Wallis Test Results

Enrolled	Obs	Rank Sum
No	21	1016.50
Yes	108	7368.50

χ^2 (with ties) $= 7.122$, $df = 1$, $p = 0.0076^{***}$

Analysis reveals significant differences in enrolment rates between BPL and non-BPL card-holders ($p < 0.01$).

Table B5

Likelihood of Enrolment by BPL Status: Logistic Regression Results

	Dependent variable:
	Enrolment
BPL Card-holder	
Yes	1.714^{**}
	(0.771)
No Response	16.336
	(1,769.258)
Constant	1.230^{***}
	(0.261)
Observations	129
Log Likelihood	−52.850
Akaike Inf. Crit.	111.699
Note:	*p<0.1; **p<0.05; ***p<0.01

BPL cardholders show significantly higher odds of enrolment ($\beta = 1.714$, $p < 0.05$), with baseline enrolment probability being positive ($\beta = 1.23$, $p < 0.01$).

Table B6

Likelihood of Enrolment by Family Size: Logistic Regression Results

	Dependent variable:
	Enrolment
Number of Family Members	
Exactly Four	0.773
	(0.546)
More than Four	1.787**
	(0.796)
Constant	1.046***
	(0.322)
Observations	129
Log Likelihood	−53.754
Akaike Inf. Crit.	113.508
Note:	*p<0.1; **p<0.05; ***p<0.01

Households with more than four members show significantly higher odds of enrolment (β = 1.787, $p < 0.05$), while households with exactly four members show no significant difference (β = 0.773, $p > 0.05$). Baseline enrolment probability is positive (β = 1.046, $p < 0.01$).

Table B7

Differences in Enrolment Rates by Number of Earning Members: Kruskal-Wallis Test Results

Enrolled	Obs	Rank Sum
No	21	970.50
Yes	108	7414.50

χ^2 (with ties) $= 8.594$, $df = 1$, $p = 0.0034^{***}$

Analysis reveals significant differences in enrolment rates between households with less than two and at least two earning members ($p < 0.01$).

Table B8

Differences in Health Discussion Frequency by Card Usage: Kruskal-Wallis Test Results

Enrollee Card Usage	Obs	Rank Sum
Did not use card	38	2519.00
Used card	70	3367.00

χ^2 (with ties) $= 8.604$, $df = 1$, $p = 0.0034^{***}$

Analysis reveals significant differences in health discussion frequencies across card usage.

Appendix C. Miscellaneous Tables & Figures

Table C1

A Breakdown of Hospitalisation Data

Characteristics	Total (%)
Number of Family Members Hospitalised	
1	122 (94.57%)
2	7 (5.43%)
Sex of Last Person Hospitalised	
Male	58 (44.96%)
Female	71 (55.04%)
Number of Days Admitted	
1 week or less	73 (56.59%)
1-2 weeks	32 (24.81%)
2-3 weeks	15 (11.63%)

4 weeks or more	9 (6.98%)

Figure C2

Enrolment Rates by Number of Earning Members

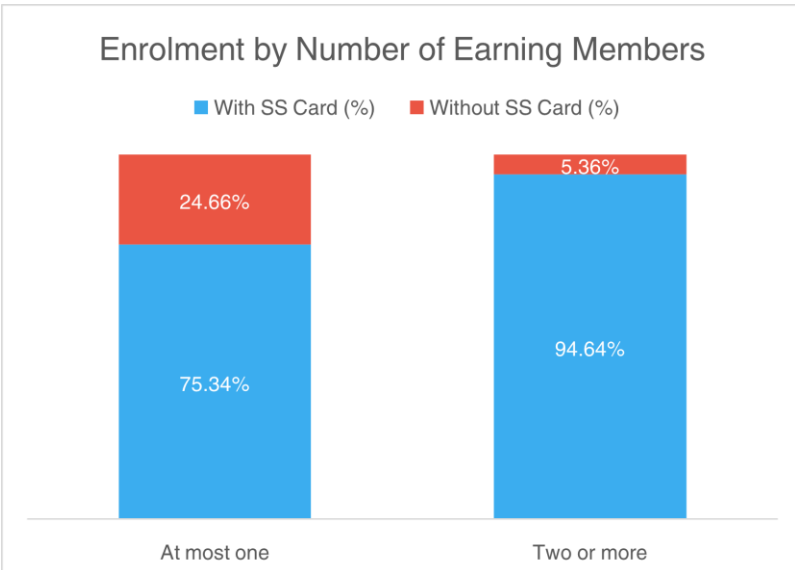


Figure C3

Enrolment Rates by Number of Aged Members

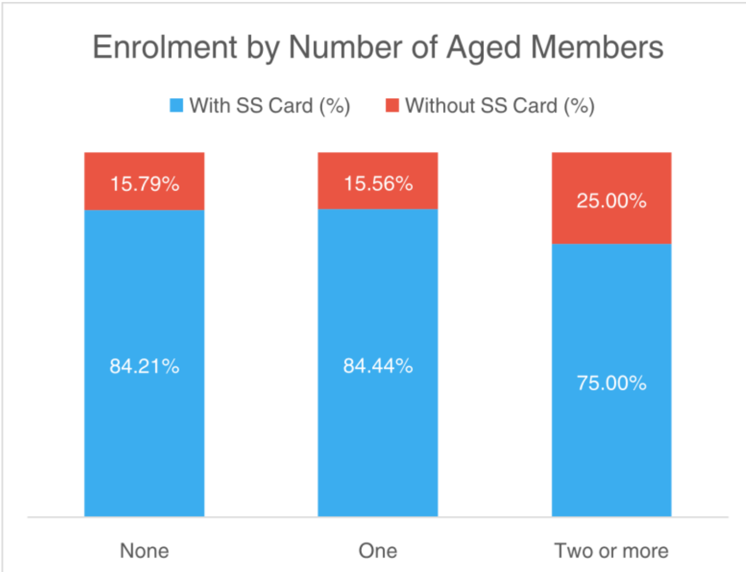


Figure C4

Enrolment Rates by BPL Status

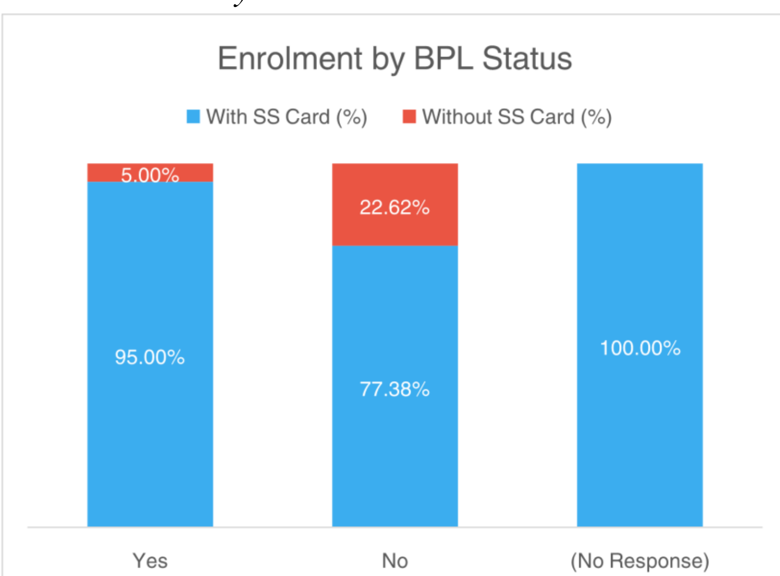


Figure C5

Reasons for not enrolling into Swasthya Sathi

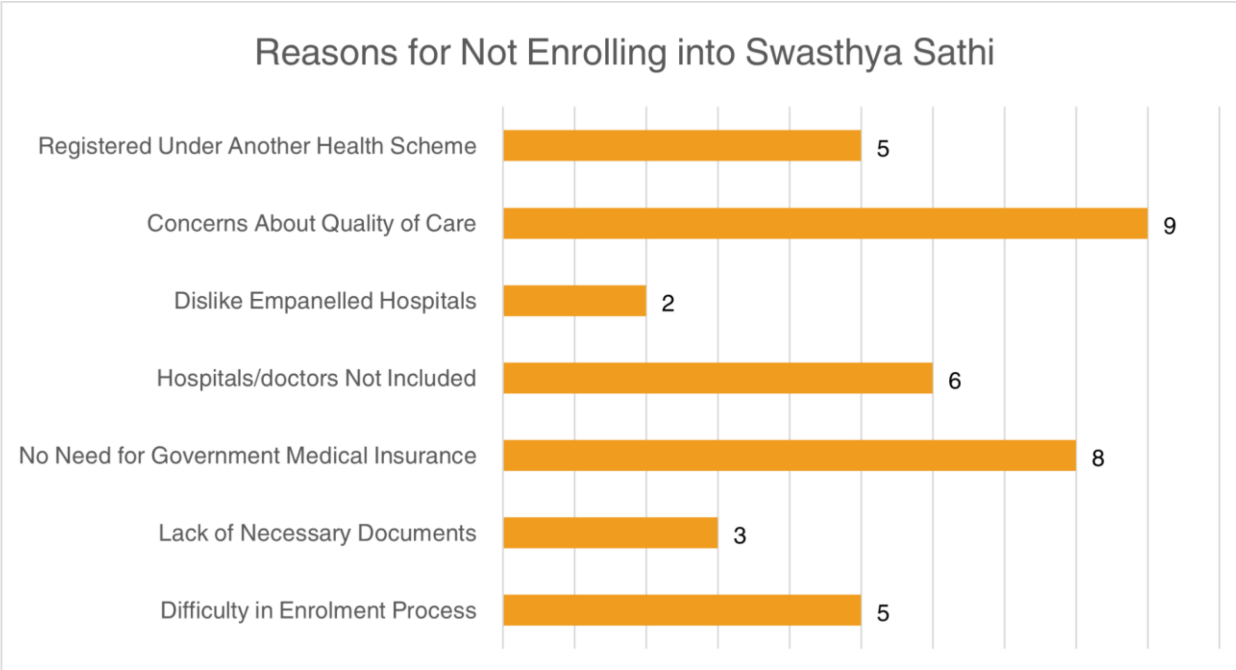


Figure C6

Reasons for non-usage of Swasthya Sathi scheme during their last hospitalisation episode